

Meridian Health Insurance

Member Handbook

Plans, Benefits, and Prior Authorization Guide

Plan Year 2026

This handbook is a synthetic, fictional document created for software testing and model training purposes. It does not represent a real insurance company, plan, or policy.

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1. About Meridian Health Insurance

Meridian Health Insurance provides health coverage across Bronze, Silver, Gold, and Platinum tiers, offered through HMO, PPO, EPO, and POS plan structures. This handbook explains what your plan covers, how prior authorization works, and what to expect if a service you need requires review before it can be approved.

Prior authorization (PA) is a process used to confirm that a requested service is medically necessary and covered under your specific plan before it is performed. It is not a guarantee of payment, and it does not replace your provider's clinical judgment -- it is a coverage check performed alongside it.

2. Plans We Offer

Plan Structures

Plan Type	Network Flexibility	Referrals Required?	Typical PA Volume
HMO	In-network only	Yes, from PCP	Higher
EPO	In-network only	No	Higher
POS	In and out-of-network (higher cost out-of-network)	Yes, from PCP	Moderate
PPO	In and out-of-network	No	Lower

Coverage Tiers

Tier	Typical Annual Deductible	Typical Out-of-Pocket Max	Plan Pays
Bronze	\$5,000 - \$8,000	\$9,000 - \$12,000	~60%
Silver	\$2,500 - \$5,000	\$6,000 - \$9,000	~70%
Gold	\$1,000 - \$2,500	\$3,000 - \$6,000	~80%
Platinum	\$500 - \$1,000	\$3,000 or less	~90%

3. Understanding Prior Authorization

When your provider submits a prior authorization request, it moves through the following stages:

- 1 Submission -- your provider submits the request with clinical documentation (diagnosis, treatment history, labs/imaging as applicable).
- 2 Policy Verification -- we confirm your policy is active and that the requested service is a covered benefit under your plan.
- 3 Clinical Review -- a reviewer (or automated review for straightforward cases) checks the request against the applicable clinical policy criteria.
- 4 Determination -- the request is Approved, Denied, or returned as Pending Additional Information if documentation is incomplete. Complex cases may be placed In Review for secondary or peer-to-peer evaluation.
- 5 Notification -- you and your provider are notified of the outcome, along with the reason and, if applicable, appeal instructions.

Review Timelines by Urgency

Urgency Level	Typical Decision Timeframe
Routine	Within 5-7 business days
Urgent	Within 24-72 hours
Emergent	Immediate / concurrent with care

4. Services That Typically Require Prior Authorization

The categories below commonly require prior authorization. Whether a specific service requires PA under your plan depends on your policy -- refer to your individual policy document for the exact list that applies to you.

- Advanced Imaging (MRI/CT)
- Specialty Drugs / Biologics
- Durable Medical Equipment
- Inpatient Surgery
- Outpatient Surgery
- Behavioral Health Procedures
- Cardiac Procedures
- Physical/Occupational Therapy
- Sleep Studies & CPAP
- Gastroenterology Infusions

5. Clinical Review Criteria Summary

The table below summarizes the clinical policies used during review. Full criteria, including complete approval and denial conditions, are published in the separate Clinical Coverage Policy Manual.

Policy #	Service	Key Requirement (summary)
MHI-ORTHO-014	Lumbar Spine MRI (without contrast)	Documented low back pain or radiculopathy present for at least 6 consecutive weeks
MHI-RHEUM-007	Biologic Disease-Modifying Antirheumatic Drugs (Addition of biologic agents)	Confirmed diagnosis of moderate-to-severe rheumatoid arthritis by a rheumatologist
MHI-ENDO-021	Continuous Glucose Monitor (CGM)	Confirmed diagnosis of diabetes mellitus (Type 1 or Type 2)
MHI-NEURO-009	CGRP Inhibitor Therapy for Chronic Migraine (off-inpatient)	Diagnosis of chronic migraine (>=15 headache days/month for >=3 months)
MHI-PULM-011	CPAP Therapy for Obstructive Sleep Apnea	Diagnostic sleep study (in-lab polysomnography or home sleep apnea test) confirming
MHI-PSYCH-018	Transcranial Magnetic Stimulation (TMS)	Confirmed Major Depressive Disorder
MHI-CARD-003	Diagnostic Cardiac Catheterization	Positive or high-risk finding on non-invasive stress testing (stress echo, nuclear stress t
MHI-ORTHO-022	Total Knee Arthroplasty (Knee Replacement)	Radiographic evidence of moderate-to-severe osteoarthritis (Kellgren-Lawrence Grade
MHI-PSYCH-005	Stimulant Medication for ADHD (Lisdexa and Amphetamine)	Confirmed ADHD diagnosis by a qualified prescriber using standardized diagnostic crit
MHI-GI-016	Biologic Infusion Therapy for Crohn's Disease (infliximab, adalimumab)	Confirmed diagnosis of moderate-to-severe Crohn's disease (via endoscopy/imaging)

6. Member Rights & Appeals Process

If a request is denied, you or your provider have the right to appeal the decision.

- 1 Internal Appeal -- submit a written appeal within 180 days of the denial notice, including any additional clinical documentation supporting medical necessity.
- 2 Peer-to-Peer Review -- your provider may request a discussion directly with the reviewing clinician before or during the appeal.
- 3 External Review -- if the internal appeal is unsuccessful and the service in question is \$500 or greater, you may request an independent external review by a third party not affiliated with Meridian Health Insurance.

7. Contact Information & Glossary

Member Services: 1-800-555-0142 (Mon-Fri, 8am-8pm)

Prior Authorization Status Line: 1-800-555-0198

Appeals Department: appeals@meridianhealthinsurance-example.com

Glossary

Medical Necessity: A service or supply that is appropriate and required to diagnose or treat a condition, consistent with accepted standards of medical practice.

Formulary: The list of drugs covered by a plan, often organized by tier.

Step Therapy: A requirement to try a lower-cost or first-line treatment before a more expensive or advanced treatment is approved.

In-Network: Providers or facilities that have a contract with Meridian Health Insurance.

EOB (Explanation of Benefits): A statement showing what was billed, what the plan paid, and what the member may owe.